

7.35.3 NMAC: proposed amendments outside the practicum. 4 of 4, Patients, applicants and process

Working draft v1, July 26, 2026. Rule hearing August 28, 2026. Not a filing, not submitted, and not adopted rule text.

- 7.35.3.8, .9, .13 and .22 through .28. Enrollment and certification applications, complaints, review of denials, and discipline.
- This is one of four documents covering the twenty-four sections of 7.35.3 NMAC that the practicum amendment draft in *amendments/* does not reach. That draft covers 7.35.3.14, .18, .19, Paragraph (5) of Subsection H of .20, and a proposed .29, and nothing here reopens any of them.
- Every proposed change carries a citation to the provision, source document, or transcript it comes from. Where a defect has no fix that a source supplies, the published text is left exactly as it stands and the question is stated at that provision, with the choices and who decides.
- No figure appears in a right column unless a source carries it. A figure taken from elsewhere in the rule is badged with where it comes from.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged.

60 from 7.35.3.15 B marks a figure taken from another provision of the same rule rather than newly drafted. A provision shown with no amendment is reproduced because it carries a finding.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request a record review to be conducted by the department, in accordance with this rule.

Subsection D, review of a denied patient application

D. A person whose application for patient enrollment is denied for failure to complete an application or failure to meet a submittal requirement of this rule may request ~~a record review to be conducted by the department~~ an informal administrative review under 7.35.3.25 NMAC, in accordance with this rule.

SOURCE Conforming amendment. 7.35.3.8 NMAC Subsection D, page 2, gives a denied patient applicant "a record review to be conducted by the department". The rule provides no proceeding called a record review. 7.35.3.25 NMAC, page 16, provides an informal administrative review for "An applicant for patient enrollment whose application has been denied", which is the same class of person. The amendment names that proceeding and cites it. The July 9, 2026 draft titled the corresponding section "DENIAL OF AN INITIAL PATIENT APPLICATION; RECORD REVIEW", and its Subsection A read that applicants "may request a record review from the department", so the two names described one proceeding in that draft. Section 7.35.3.8 is new in the published rule.

Please review. Finding M23 of `analysis/july23-rule-concerns.md`. Three provisions describe review of a denial, and their scopes do not line up. This subsection covers a patient applicant denied for an incomplete application or a missed submittal requirement. Subsection A of 7.35.3.25 NMAC covers any patient applicant whose application has been denied. Paragraph (7) of Subsection C of 7.35.3.27 NMAC gives a hearing to an applicant denied for any reason other than an incomplete application or a missed submittal requirement. Read together, the informal review reaches every denial and the hearing reaches every denial except the two that this subsection covers, so a patient applicant denied on the merits may have both. The amendment drafted here only fixes the name of the proceeding and leaves the overlap alone. Two choices on the overlap. Narrow Subsection A of 7.35.3.25 NMAC to the grounds this subsection names, or state that the informal review and the hearing are alternatives and which one a patient applicant elects. Department of Health staff and department counsel decide, because the answer affects what process a denied applicant is owed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply within six months. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

Paragraphs (5) and (6) of Subsection A, re-application after denial

(5) If the application is denied, the applicant may re-apply ~~within~~ **no earlier than** six months **after the date of the denial**. (6) An applicant who is denied a second time may not re-apply for six months from the denial.

SOURCE Conforming amendment. Same defect and same fix as 7.35.3.10 NMAC Subsection C. 7.35.3.9 NMAC Paragraphs (5) and (6) of Subsection A, page 2.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; (4) Documentation of HIPAA certification completed within the preceding two years; (5) Completed W-9 form; and (6) Such additional information as the department may reasonably require.

Subsection D, certifying clinician application requirements

D. Certifying clinician application requirements: An applicant for certification as a certifying clinician shall additionally submit the following, using the electronic system designated by the department: (1) Documentation of current professional license to practice in New Mexico (e.g. MD, NP); (2) NM controlled substance number; ~~(4)~~ **(3)** Documentation of HIPAA certification completed within the preceding two years; ~~(5)~~ **(4)** Completed W-9 form; and ~~(6)~~ **(5)** Such additional information as the department may reasonably require.

SOURCE Correction. Finding N3 of `analysis/july23-rule-concerns.md`, verified against the published rule. The paragraphs of 7.35.3.9 NMAC Subsection D, page 3, run (1), (2), (4), (5), (6). There is no (3). The amendment renumbers so that the list is consecutive and adds no item. The paragraph reproduced here contains the controlled substance number requirement at Paragraph (2), which is out of scope for this document by decision; that paragraph is reproduced as published and is not amended.

Please review. The renumbering assumes that the gap is a numbering error rather than the trace of a deleted requirement. Section 7.35.3.9 has no counterpart in the July 9, 2026 draft, so this record cannot show what, if anything, stood at Paragraph (3). Two choices. Renumber as drafted, or restore the missing requirement if one was intended. Department of Health staff decide, because only the department knows what the paragraph held.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading

7.34.3.13 REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

Section heading

~~7.34.3.13~~ **7.35.3.13** REQUIREMENTS AND PROHIBITIONS FOR CERTIFYING CLINICIANS, PRACTITIONERS, AND FACILITATORS:

SOURCE Correction only. Finding N1 of `analysis/july23-rule-concerns.md`, verified against the published rule. The heading at page 8 reads 7.34.3.13; the history note at the end of the same section reads “[7.35.3.13 NMAC, xx/xx/2026]”. Chapter 34 of Title 7 is a different chapter. Five headings in the rule read 7.34.3: those of 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.23 and 7.35.3.25. The headings of 7.35.3.14 and 7.35.3.20 are corrected in the practicum amendment document; the other three are corrected here.

Subsection B, facilitators; scope of work

B. Facilitators; scope of work: A facilitator is authorized to work alongside a practitioner during medical psilocybin services. A facilitator works under the direct supervision of a practitioner, and provides peer support to qualified patients, as well as logistical and administrative support to the practitioner and to the healing center. A facilitator shall not perform any patient care outside this scope, unless another license held by the facilitator permits it.

Subsection B, facilitators; scope of work

Not amended. This provision is reproduced because it carries a finding recorded below.

SOURCE Not amended. Recorded because it is the source of the facilitator definition drafted at 7.35.3.7 NMAC and because of finding M15 of `analysis/july23-rule-concerns.md`. 7.35.3.13 NMAC Subsection B, page 8. Carried over from the July 9, 2026 draft.

Please review. Finding M15 of `analysis/july23-rule-concerns.md`. This subsection puts a facilitator under the direct supervision of a practitioner and limits the facilitator to peer support and to logistical and administrative support. Three provisions sit against it, and all three are in the practicum amendment document rather than this one. Subsection B of 7.35.3.14 NMAC, page 9, lets a facilitator possess psilocybin products and provide them to qualified patients. Paragraph (5) of Subsection H of 7.35.3.20 NMAC, page 14, permits a group session staffed by one facilitator or qualified student for every two patients with one practitioner for every eight patients, so a facilitator may be the only certificant with a given patient. Subsection C of 7.35.3.19 NMAC, page 13, has practitioners supervising facilitators during administration sessions as part of the practicum. Whether “direct supervision” means presence in the room, presence at the location, or availability is not stated anywhere in the rule. Three choices. Define direct supervision in this part, which the definitions drafted at 7.35.3.7 NMAC could carry; narrow the facilitator authority in the three provisions named above to match this subsection; or widen this subsection to match them. No source in this record chooses among the three, so none is drafted. The Training and Education Committee and Department of Health staff decide the meaning, and because the conflicting provisions belong to the practicum draft, any amendment has to be coordinated with that document rather than made here.

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, dual ownership 7.34.3.23 PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee.	Entire section, dual ownership 7.34.3.23 7.35.3.23 PROHIBITION AGAINST DUAL OWNERSHIP IN CERTIFICANT AND PERMITTEE: A certificant or a person who holds an ownership interest in a certificant shall not hold an ownership interest in a permittee. ----- SOURCE Correction only. Finding N1. The heading at page 15 reads 7.34.3.23; the history note at the end of the same section reads “[7.35.3.23 NMAC, xx/xx/2026]”. The substance is not amended. “Permittee” is defined at 7.35.2.7 NMAC as a psilocybin producer or psilocybin testing laboratory that holds a permit, so the prohibition operates on the producer and laboratory side without further definition. Carried over from the July 9, 2026 draft.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. certified mail or through the electronic system designated by the department electronic system. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

Entire section, complaints to the department

7.35.3.24 COMPLAINTS TO THE DEPARTMENT: A qualified patient or certificant may submit a complaint to the department regarding a certificant in the medical psilocybin program, or regarding any activity or concern related to the medical psilocybin program. The complaint should be in writing, addressed to the medical psilocybin program, and submitted by U.S. ~~certified~~ mail or through the electronic system designated by the department ~~electronic system~~. The department may request additional information for purposes of its own inquiry. A complaint shall include the name and contact information for the complainant, which shall remain confidential; and a statement of the nature of the complaint and any individuals who may be involved. The department may, within its discretion, choose to conduct a subsequent inquiry, and may request additional information from the complainant for this purpose.

SOURCE Three changes, all restorations or corrections. Confidentiality: finding M17 of `analysis/july23-rule-concerns.md`, verified against the July 9, 2026 draft, which required that a complaint include the "Name and contact information for the complainant which will remain confidential". The published section, page 15, requires the name and contact information and drops the assurance. Mail: the July 9, 2026 draft read that a complaint "should be in writing and submitted by U.S. mail or through the electronic" system; the published section requires "U.S. certified mail". Duplication: the published section reads "through the electronic system designated by the department electronic system". Section 7.35.3.24 is new in the published rule; the July 9, 2026 draft carried the same substance as an unnumbered list headed "Complaint Reporting".

Please review. Two questions the amendment does not reach. Standing: the July 9, 2026 draft provided that "Patients or staff may lodge a complaint with the department." The published section allows a complaint from a qualified patient or a certificant, so an employee of a healing center who is neither has no route under this section. Two choices. Restore standing for staff, or leave the section as published and rely on the adverse health event reporting in Subsection L of 7.35.3.20 NMAC. Scope of the confidentiality restored here: the July 9 wording protected the complainant's name and contact information and said nothing about disclosure to the certificant complained of, or about the application of the Inspection of Public Records Act. Department of Health staff and department counsel decide both, and the board should be told whether a complainant can be assured of confidentiality before the rule is filed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Section heading and Subsection A

7.34.3.25 INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT APPLICATIONS:

A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

Section heading and Subsection A

~~7.34.3.25~~ **7.35.3.25** INFORMAL ADMINISTRATIVE REVIEW OF DENIED PATIENT APPLICATIONS: A. Administrative review: An applicant for patient enrollment whose application has been denied may request an informal administrative review from the department.

SOURCE Correction only. Finding N1. The heading at page 16 reads 7.34.3.25; the history note at the end of the same section reads “[7.35.3.25 NMAC, xx/xx/2026]”. The substance is not amended.

Paragraph (1) of Subsection D, content and timeline

(1) Content and timeline: The administrative review shall be completed, and a decision setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for a record review.

Paragraph (1) of Subsection D, content and timeline

(1) Content and timeline: The administrative review shall be completed, and a decision shall be issued setting forth the reasons for the decision and the evidence upon which the decision is based, no later than 15 calendar days from the date that the program receives the written request for ~~a record review~~ an informal administrative review.

SOURCE Two corrections. Grammar: finding N4 of `analysis/july23-rule-concerns.md`, verified against the published rule. The sentence at page 16 has no verb governing the decision, and is carried over verbatim from the July 9, 2026 draft. The amendment supplies the verb and changes nothing else. Terminology: the same sentence refers to “the written request for a record review”, while Subsection A of the same section and Paragraph (1) of Subsection B call the proceeding an administrative review. The 15 calendar days are not amended.

Please review. Two questions on this section that the corrections do not reach. The administrative review committee: Subsection C of this section and Paragraph (2) of Subsection D assign the review to “the administrative review committee”, and nothing in 7.35.3 NMAC or 7.35.2 NMAC constitutes that committee, states who sits on it, or says how it is appointed. The term appears four times in the rule, all in this section, and it is carried over from the July 9, 2026 draft. Judicial review: Subsection E provides that “Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee.” and the Wilson working redline comments on that subsection “they really went in the opposite direction of due process here...” Neither is drafted, because constituting a committee and limiting judicial review are both beyond what a correction can carry, and no source in this record proposes terms for either. Department of Health staff must say who the committee is, and department counsel must say whether Subsection E is within the department’s authority. The board should have both answers before the rule is filed.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a certified patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

Paragraph (1) of Subsection C, persons who may request a hearing

C. Persons who may request a hearing: The following persons may request a hearing to contest an action or proposed action of the department, in accordance with this rule: (1) a ~~certified~~ qualified patient; (2) a certified practitioner; (3) a certifying clinician who was certified by the department; (4) a certified facilitator; (5) a certified healing center or other approved location; (6) a certified educational program; and (7) an applicant for enrollment or certification as any of the foregoing, whose application is denied for any reason other than failure to submit a completed application or failure to meet a submittal requirement of this rule.

SOURCE Conforming amendment. Paragraph (1) of Subsection C, page 17, reads "a certified patient". A patient is enrolled rather than certified: 7.35.3.8 NMAC Subsection E, page 2, provides for "A qualified patient's enrollment", 7.35.3.22 NMAC, page 15, distinguishes "a qualified patient or certificant", and 7.35.2.7 NMAC defines "qualified patient". The subsection is reproduced in full because it is the source of the certificant definition drafted at 7.35.3.7 NMAC. Carried over from the July 9, 2026 draft.

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the clinician has engaged in unprofessional or dishonorable conduct;

Paragraph (8) of Subsection B, grounds for disciplinary action

(8) for certifying clinicians and practitioners: any determination by the individual's licensing body that the ~~clinician~~ certifying clinician or practitioner has engaged in unprofessional or dishonorable conduct;

SOURCE Correction. Paragraph (8) of Subsection B, page 17, opens "for certifying clinicians and practitioners" and then refers only to "the clinician". The amendment names both classes the paragraph applies to. Carried over from the July 9, 2026 draft.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Subsection M, continuances

M. Continuances: The hearing examiner may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

Subsection M, continuances

M. Continuances: The hearing ~~examiner~~ **officer** may grant a continuance for good cause shown. A motion to continue a hearing shall be made at least 10 calendar days before the hearing date.

SOURCE Correction. Finding N4. Subsection M, page 18, reads “The hearing examiner may grant a continuance for good cause shown”. Every other subsection of 7.35.3.27 NMAC that names the presiding official calls that official the hearing officer, including Paragraph (1) of Subsection E, page 17: “All hearings held pursuant to this section shall be conducted by a hearing officer appointed by the secretary.” The 10 calendar days are not amended. Carried over from the July 9, 2026 draft.

Please review. Finding N5 of `analysis/july23-rule-concerns.md`, which the correction to this subsection does not reach. Subsection A of this section newly allows the department to suspend a certification immediately. After a suspension, Subsection F allows the hearing to be held up to 60 calendar days after the request, Paragraph (2) of Subsection O allows the hearing officer 30 calendar days after the last submission to recommend, and Paragraph (3) of Subsection O allows the secretary 45 calendar days after receipt to decide, so a suspended certificant can be out of practice for more than 135 days before a final decision. The rule provides no expedited track. Two choices. Add an expedited schedule for a hearing that follows an immediate suspension, or leave the timeline as published. No source in this record supplies a shorter period, so none is drafted. Department of Health staff and department counsel decide, because the periods bind the department and the secretary.

Addendum A Terms this part uses and no part defines

7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Every count in the table below is produced from the text layer of the published rule and of 7.35.2 NMAC each time this document is built, section by section, so no count in it is transcribed by hand. Column headings are section numbers within each part. The first block is terms used in 7.35.3 NMAC that 7.35.2.7 NMAC does not define. The second block, shaded, is the terms 7.35.2.7 NMAC does define, shown for contrast.

Addendum A

TERMS THIS PART USES AND NO PART DEFINES

Term in 7.35.3 NMAC	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	Part 3	Part 2
educational program	1					1				28		7			3	3	13	1	2	1	3					1	3		68	0
facilitator	1					1			5	5	3	1	14	5			2	15	4	3						1	4		65	0
healing center	1										10	1	2	10					1	21	3					1	3		54	0
certifying clinician						1		16	5	4			11					10								1	4		53	0
other approved location	1										6		1	3					1	14	3					1	3		34	0
practicum									3	4		1			1		5		13	2									29	0
student												5				3	12		5	3									28	0
certificant																					2	3	3	2			1		11	0
registrant																				10									10	0
didactic										4								4	2										10	0
adverse health event											1		1							4									6	0
administrative review committee																									4				4	0
graduate												1					2		1										4	0
simulated patient																	1	2	1										4	0
medical psilocybin services													1					1		1									3	0
equity and access fund								1																					1	0
Defined in 7.35.2.7 NMAC, for contrast																														
practitioner	1					1			5	5	3	1	12	6			2	14	6	3						1	5		66	12
clinician	1						5						4												1		2		13	4
qualified patient						1		3			2		4	3						3		3		1		1	1		22	6
administration session											5			3				1		7		1							17	4
approved location	1					1					6	1	1	3					1	15	3					1	3		37	2
guide																													0	3

What the table shows. Sixteen terms carrying regulatory consequence in this part are defined in neither part. Four of them are defined in the draft at 7.35.3.7 NMAC, because a single provision of the published rule supplies the content: certificant, facilitator, registrant of another approved location, and student. Twelve are not, and the two largest are healing center and certifying clinician. "Guide", the one defined term in 7.35.2.7 NMAC for an individual who assists practitioners during administration sessions, appears nowhere in 7.35.3 NMAC, while "facilitator", which 7.35.2.7 NMAC does not define, appears throughout it. 7.35.2 NMAC was adopted effective June 23, 2026, so amending 7.35.2.7 NMAC is a separate rulemaking and any term this part needs has to be carried in this part.

The Act. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. 7.35.2.7 NMAC defines "Clinician" in the same terms except that it reads certification where the Act reads permit, and defines "Permit" as the authorization to operate as a psilocybin producer or psilocybin testing laboratory. Section 5 of the Act exempts a producer, a clinician and a qualified patient from arrest, prosecution and penalty. Neither the Act nor 7.35.2.7 NMAC uses the word facilitator. The consequence is stated at 7.35.3.7 NMAC in the redline above.

Addendum B Dependencies across the two drafting scopes

Provisions in these four documents that operate on, or are operated on by, a provision the practicum amendment draft in *amendments/* reaches. Nothing in these documents amends a provision in that draft's scope. Where a fix would require one, it is recorded here and left undrafted.

Addendum B

DEPENDENCIES ACROSS THE TWO DRAFTING SCOPES

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.11 A(11), page 5	7.35.3.14 C, page 9	7.35.3.14 C conditions healing center owner and employee authority on being registered with the department, and no registration exists. The registration is drafted here, at the point where the healing center already files the names.	Drafted here. The practicum draft flagged 7.35.3.14 C and did not amend it, and it is not amended here either.
7.35.3.7, page 1	7.35.3.20 H(5), page 14	The definition of student drafted here is taken from the qualified student test in 7.35.3.20 H(5). The practicum draft amends that paragraph for the permit title and does not change the test.	Drafted here, sourced there. If the test changes, the definition follows it.
7.35.3.7, page 1	7.35.3.14 B, page 9	7.35.3.14 B authorizes facilitators to possess psilocybin products and provide them to qualified patients. Whether a facilitator is a clinician under Section 3(B) of the Medical Psilocybin Act, and so within the Section 5 exemption, governs whether that authority holds. This is the administering half of Paragraph (2) of Subsection B of Section 5 of the Act. The practicum draft reads the taking half of the same provision against Subsection H of Section 3, and its proposed 7.35.3.29 states the statutory amendment that half would need and conditions the section on it, so the two drafts read the same provision consistently.	Not drafted. Recorded at 7.35.3.7 as a question for department counsel.
7.35.3.7, page 1	7.35.3.14 A and C, page 9, with 7.35.3.20 D, page 14	The position of a student is not in doubt and is not raised in these documents. The practicum draft locates the provision of the medicine in the practitioner or the healing center, and 7.35.3.20 D provides that "students completing their practicums may be present during an administration session". The definition of student drafted here does not disturb that.	Settled in the practicum draft. Nothing here contradicts it.
7.35.3.13 B, page 8	7.35.3.14 B, 7.35.3.19 C, 7.35.3.20 H(5)	The facilitator scope of work in 7.35.3.13 B is narrower than the authority the other three provisions confer, and "direct supervision" is not defined anywhere in the rule.	Not drafted. Any amendment has to be coordinated with the practicum draft.
7.35.3.17 A, pages 10 to 11	Proposed 7.35.3.19 H, which exists only in that draft's re-lettering of 7.35.3.19	Whether the consultation requirement is stated in hours or in cases, and whether it sits at the proposed 7.35.3.19 H or at 7.35.3.17 A, is open in the practicum document for Dr. Metz, Ms. Wilson and Dr. Leeman. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H.	Not drafted. 7.35.3.17 A is left exactly as published so that their answer governs.
7.35.3.10 D(1), page 5	7.35.3.19 G(4), page 13, which that draft re-letters as 7.35.3.19 K	Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027. For the same 40 hours of contact time, 7.35.3.10 D(1) requires "two separate group sessions" and 7.35.3.19 G(4) requires "A minimum of one group session". The practicum draft records the conflict at its 7.35.3.19 K and amends neither provision, on the ground that 7.35.3.10 is outside its scope.	Not drafted here either. Conforming one to the other is a choice between two figures already in the rule, so neither document makes it and the conflict stands until the committee chooses.
7.35.3.16 A and C, page 10	7.35.3.18, pages 11 to 12	The third-party evaluation assesses the curriculum that 7.35.3.18 requires. Curing the evaluator conflict does not touch the curriculum, and the practicum draft's changes to the curriculum do not touch the conflict.	Drafted here. No coordination needed.
7.35.3.20 heading, page 13	7.35.3.20 heading, page 13	The heading reads 7.34.3.20. The practicum draft corrects it.	Not duplicated here. The correction stands in the practicum draft.

Addendum C Defects recorded and not drafted, and why

Every defect this record found in the twenty-four sections, that this draft does not amend. The reason in each row is the reason no language is proposed, not an assessment of severity. Each has a review note at the provision named, stating the choices and who decides.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.5, page 1, with the history notes at page 19	Twenty-six history notes carry the placeholder "xx/xx/2026"; the notes for 7.35.3.27 and 7.35.3.28 carry 9/22/2026. Under 7.35.3.5 a later date cited at the end of a section controls, so two sections would take effect before the rest has a date.	No source supplies the intended effective date, and which way to conform is the department's to choose.
7.35.3.7, page 1	Twelve of the sixteen undefined terms have no definition in any source, including healing center, 54 occurrences, and certifying clinician, 53 occurrences.	Drafting a defined term with regulatory consequence from no source would be composing it. Addendum A records the slate.
7.35.3.9 B, page 3	A renewal application is due no less than 30 days before expiry, and the department has no deadline to decide it, so a certification can lapse while a timely renewal is pending. Nothing continues the certification in the interim.	The fix is a continued-validity provision, which is a new grant, and no source in this record proposes its terms.
7.35.3.9 E(1) and F, page 3	A practitioner applicant must document a professional license "(e.g. PSY, LSW, LCSW)" with no stated scope of practice, and a facilitator applicant needs no license at all. The Wilson working redline comments "Also no scope here - what happened to therapy scope? It's missing - if DOH wants that, they need to include it." and "without specifying, they're setting themselves up to have athletic trainers and massage therapists apply and then not have a regulation to point to explain why that's not sufficient."	The redline raises the gap and supplies no list of qualifying licenses, and no other source in this record supplies one.
7.35.3.8 C, page 2, with 7.35.3.9 A(1), page 2	The department has 30 business days to decide a patient application and 30 calendar days to decide a certificant application.	Conforming one to the other is a choice between two figures already in the rule, and both bind the department.
7.35.3.10 A(2) and A(3), pages 4 to 5	The list of out-of-jurisdiction programs populates only when an individual application relying on a program is approved, so it is empty at launch. A(3) refers to programs approved by Oregon and Colorado "as of December 31, 2027", which is after the December 31, 2027 waiver in D(1) has closed.	Both fixes are new grants of authority to the department, and no source in this record proposes their terms.
7.35.3.10 D(1), page 5, with 7.35.3.19 G(4), page 13	The two 40-hour waivers set different group-session minimums, two against one.	Conforming one to the other is a choice between two figures already in the rule. The practicum draft records the same conflict and amends neither provision, so neither document makes the choice. Addendum B.
7.35.3.11 A(22)(d) and (e), and B(10) (d), pages 5 to 7	The 15-minute threshold for a remote natural environment. The Wilson working redline asks twice whether it should be 30.	A question in the source, not a recommendation, and no source supplies a figure.
7.35.3.11, page 5	The two-person universal requirement the Wilson working redline adds, which its author records did not reach the published rule.	The author records that she does not know where the provision belongs. Placing it would answer her question.
7.35.3.12 A(20) and B(1), pages 7 to 8	An application filed on or before December 31, 2027 may be approved without the third-party evaluation provided the evaluation is submitted by December 31, 2027, so the deferral shortens to nothing as that date approaches and is unavailable on the last day.	The fix is a period, and no source in this record supplies one. Tying it to the certification term would be a choice about how long a program may operate unevaluated.
7.35.3.12, pages 7 to 8, against the July 9, 2026 draft	The grounds for denying an educational program application present in the July 9 draft do not appear in the published rule. Finding M9.	Restoring deleted grounds is a substantive policy decision, and this record cannot show whether the deletion was deliberate.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.17 A, pages 10 to 11	The mentoring obligation, and whether it is stated in hours or cases and where it sits.	Open in the practicum document for three named people. Addendum B.
7.35.3.17 B, page 11	The test-out price cap is a fraction of the price of "each of the educational modules", which assumes per-module pricing. The Wilson working redline strikes the subsection.	Whether the option survives at all is the committee's decision, and a restated cap is worth drafting only if it does.
7.35.3.20 A, page 13, with 7.35.3.21 A, page 15	Healing centers must keep a roster of all qualified patients intended to use and who have used the location, and the department may interview patients.	Whether the roster should be kept in a form that does not identify patients turns on the department's data needs, which no source in this record states.
7.35.3.20, page 13	The term "registrant of another approved location", 10 occurrences, against the certification that 7.35.3.11 B issues.	The definition drafted at 7.35.3.7 ties the two words together. Conforming all 10 occurrences instead is a drafting choice, and this document does not make it.
7.35.3.21, page 15	No interval is set for department assessments, and none for repeating a third-party evaluation after the first.	An interval is a figure and a demand on department capacity. No source supplies either.
7.35.3.25 C and D(2), page 16	The administrative review committee decides a denied patient application and is constituted nowhere in 7.35.3 NMAC or 7.35.2 NMAC. Four occurrences, all in this section.	Only the department can say who the committee is.
7.35.3.25 E, page 16	"Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." The Wilson working redline comments on it.	Whether the provision is within the department's authority is a legal question for department counsel.
7.35.3.24, page 15	The July 9, 2026 draft allowed a complaint from patients or staff; the published section allows one from a qualified patient or certificant, so staff who are neither have no route.	Restoring standing for staff is a policy decision. The confidentiality assurance the July 9 draft carried is restored in the redline above, because that text exists in a source.
7.35.3.27, pages 16 to 19	A certificant suspended immediately under Subsection A can be out of practice more than 135 days before a final decision, and there is no expedited track. Finding N5.	An expedited schedule is a set of periods binding the department and the secretary, and no source supplies them.
7.35.3.8 D, 7.35.3.25 A, 7.35.3.27 C(7), pages 2, 16 and 17	Three provisions describe review of a denial and their scopes overlap, so a patient applicant denied on the merits may have both an informal review and a hearing. Finding M23.	The redline fixes the name of the proceeding only. Which track applies to which denial is a question of what process is owed.

Out of scope by decision. The controlled-substance number requirement for certifying clinicians. It appears at Paragraph (3) of Subsection B of 7.35.3.8 NMAC, page 1, and at Paragraph (2) of Subsection D of 7.35.3.9 NMAC, page 3. Paragraph (2) is reproduced in the redline above because the renumbering of the paragraphs around it required it. It is reproduced as published and is not amended, and nothing is proposed about it.

Addendum D Mechanical defects across all twenty-eight sections

Numbering, dates and drafting form, checked across the whole of 7.35.3 NMAC rather than only the sections these documents amend. Every count and every entry is produced from the text layer of the published rule each time this document is built.

Addendum D

MECHANICAL DEFECTS ACROSS ALL TWENTY-EIGHT SECTIONS

Item	Count	Where	Corrected
Section headings reading 7.34.3 instead of 7.35.3	5 of 28	7.34.3.13, 7.34.3.14, 7.34.3.20, 7.34.3.23, 7.34.3.25	7.35.3.13 in the document 4 above; 7.35.3.14 in the practicum draft; 7.35.3.20 in the practicum draft; 7.35.3.23 in the document 4 above; 7.35.3.25 in the document 4 above
History notes carrying a real effective date rather than a placeholder	2 of 28	7.35.3.27 reads 9/22/2026, 7.35.3.28 reads 9/22/2026	Not corrected. Recorded at 7.35.3.5 above.
History notes omitting the "- N" new-section designator	9 of 28	7.35.3.11, 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.22, 7.35.3.23, 7.35.3.24, 7.35.3.25, 7.35.3.26	Not corrected. Mechanical, and the designator is the department's to set.
Numbering gap inside a subsection	1	7.35.3.9 D runs (1), (2), (4), (5), (6). There is no (3).	Renumbered in document 4 above.
Subsections lettered "(A)" rather than "A."	4	7.35.3.11 A, and 7.35.3.14 A, B and C. Letters found: A, B, C	7.35.3.11 A in document 3 above. 7.35.3.14 in the practicum draft.
Sentence with no verb governing the decision	1	7.35.3.25 D(1), page 16. Carried over from the July 9, 2026 draft.	Corrected in document 4 above.
Sentence that does not complete	1	7.35.3.18 F, page 12.	In the practicum draft's scope. Corrected there.
Presiding official named inconsistently	1	7.35.3.27 M reads "hearing examiner"; the rest of 7.35.3.27 reads "hearing officer".	Corrected in document 4 above.
Duplicated words	1	7.35.3.24 reads "the electronic system designated by the department electronic system".	Corrected in document 4 above.
Spacing inside a heading	1	7.35.3.16 C reads "Conflict of -interest prohibitions". The July 9, 2026 draft read "Conflict-of-interest prohibitions".	Corrected in document 2 above.
Class named in the lead-in but not in the operative words	1	7.35.3.27 B(8) opens "for certifying clinicians and practitioners" and then refers only to "the clinician".	Corrected in document 4 above.
Patient described as certified rather than enrolled	1	7.35.3.27 C(1) reads "a certified patient".	Corrected in document 4 above.

Sources. Rule as published: *docs/documents/rules-draft-2026-07-23-published.pdf*, 19 pages, 7.35.3.1 through 7.35.3.28. Prior board-meeting draft, used for new against carried-over determinations: *docs/documents/rules-draft-2026-07-09.pdf*. Defined terms: 7.35.2 NMAC as adopted effective June 23, 2026, at *source-text/7.35.2-NMAC-adopted-2026-06-23.txt*. Statute: the Medical Psilocybin Act, Senate Bill 219, 57th Legislature, First Session, 2025, as passed; Section 1 provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act, and those section numbers are the ones cited here. This record does not verify any NMSA 1978 section number. Working redline: Denali Wilson, NMAC 7.35.3, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION."; no speaker is named from either transcript in this document. Inventory relied on and re-verified: *analysis/july23-rule-concerns.md*.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and hyphenation introduced by a line break rejoined. Nothing else is

altered. Every block was checked against the text layer of the published rule by exact contiguous match, and the build aborts if any block fails. *amendments-remainder/audit.py* re-checks every verifiable claim in these documents and exits non-zero on any failure; *amendments-remainder/AUDIT.md* is its output.

Status. Working draft v1, document 4 of 4. Not a filing, not submitted, and not adopted rule text. Nothing in *amendments/* or *docs/* was modified by this work.